

# NURSING: HOW WE THINK ABOUT VALUE

A transparent methodology for calculating return on investment

## THE CONTEXT

Nursing value is fundamentally different from physician-focused ROI. Nurses don't bill for services, so there's no direct revenue tied to their documentation. Value shows up in labor efficiency—overtime reduction, agency spend, and retention—plus the harder-to-measure benefits of more time at the bedside.

## VALUE MECHANISMS

### Overtime Reduction

Time saved on documentation can reduce overtime hours. We convert saved time using the OT conversion rate, applying 1.5x hourly rate for OT hours eliminated.

Directly Measurable

### Retention Savings

Documentation burden is a key burnout driver. Reducing burden may improve retention, avoiding replacement costs of \$40-60K per nurse.

Indirectly Attributable

### HAPI Prevention

Real-time documentation enables earlier visibility of pressure injury risk, supporting prevention protocols.

Indirectly Attributable

### Falls Prevention

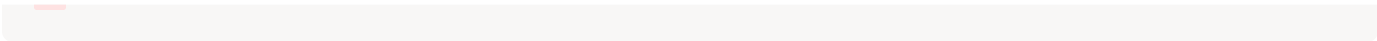
Better documentation of mobility status and risk factors supports fall prevention programs.

Indirectly Attributable

### Patient Experience (HCAHPS)

More bedside time correlates with patient satisfaction, though HCAHPS is influenced by many factors.

Qualitative



KEY ASSUMPTIONS

| Assumption           | Range           | Default     | Source                 |
|----------------------|-----------------|-------------|------------------------|
| Time saved per shift | 15-30 min       | 20 min      | Abridge customer data  |
| OT conversion rate   | 15-40%          | 25%         | Implementation studies |
| Nurse turnover rate  | 15-25%          | 18%         | NSI Nursing Solutions  |
| Burnout-related %    | 30-50%          | 40%         | ANA studies            |
| Replacement cost     | \$40-60K        | \$50K       | NSI 2023 Report        |
| HAPI rate            | 1.5-3.5/1K days | 2.5/1K days | AHRQ benchmarks        |
| Prevention rate      | 3-10%           | 5%          | Conservative estimate  |

WHAT WE CAN MEASURE

Directly Measurable:

- Overtime hours before/after
- Time-motion documentation time
- Shift completion rates

Indirectly Attributable:

- Turnover rates (many factors)
- HAC rates (clinical practice matters)
- Patient satisfaction (dozens of factors)

VALIDATION PATH

1. Baseline OT and agency spend on pilot units
2. Survey nursing staff on documentation burden
3. Track time-motion studies pre/post implementation
4. Compare turnover rates on Abridge vs. control units
- 5.

Monitor HAC and HCAHPS trends (directional only)

*"The goal isn't to prove our model right. It's to build your organization's understanding of what ambient documentation actually delivers in your context. Adjust the model based on what you learn."*